

SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

Session: Revision Mock Set 02	Duration: 3 hours
Specialty: Psychotherapy	Coefficient: 04
Eliminatory mark: < 8/20	Training reference: VTI-AEDA examination style

COMPREHENSIVE VOCATIONAL PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 02

Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.

How to use this marking guide

For revision, first attempt the paper without notes. Then compare not only the answer letter but the clinical cue that makes the answer correct. For legal/ethical items, the guide follows the concepts emphasized by the supplied course/examination materials; in real practice, applicable Cameroon law, institutional policy and the current professional code must also be checked.

PART ONE: PSYCHOPATHOLOGY - ANSWERS

1. During assessment, Owen reports persistent psychotic illness with delusions, hallucinations and/or disorganized speech or behavior, often with social or occupational dysfunction. Which of the following is the BEST match?

- A. Schizoid personality disorder
- B. Manic episode
- C. Schizophrenia**
- D. Stimulant intoxication

Answer: C - Schizophrenia. Schizophrenia is a chronic psychotic disorder; diagnosis depends on characteristic symptoms, duration and functional impact, not on a laboratory test alone.

Memory aid: 3D psychosis: Delusions, perceptual Disturbance, Disorganization.

2. Esther, a 38-year-old accountant, presents with sudden onset of delusions, hallucinations or disorganized speech/behavior lasting at least a day but less than a month, with return toward baseline. Which diagnosis or clinical concept best fits this presentation?

- A. Borderline personality disorder
- B. Brief psychotic disorder**
- C. Schizoid personality disorder
- D. Autism spectrum disorder

Answer: B - Brief psychotic disorder. The very short duration and return toward premorbid functioning distinguish brief psychotic disorder from longer psychotic disorders.

Memory aid: BRIEF = psychosis Below one month.

3. Kemi, a 44-year-old farmer, presents with significantly low body weight, intense fear of weight gain and restriction without recurrent binge-purge episodes. Which diagnosis or clinical concept best fits this presentation?

- A. Social anxiety disorder
- B. Anorexia nervosa, restricting type**
- C. Agoraphobia
- D. Schizoid personality disorder

Answer: B - Anorexia nervosa, restricting type. Anorexia nervosa requires significantly low weight plus fear of gaining weight or persistent behavior interfering with weight gain and body-image disturbance.

Memory aid: ANOREXIA: restriction + low weight + fear of weight gain.

4. During assessment, Esther reports significantly low body weight with anorexia features plus recurrent binge-eating or purging. Which of the following is the BEST match?

- A. Anorexia nervosa, binge-eating/purging type**

- B. Social anxiety disorder
- C. Brief psychotic disorder
- D. Panic disorder

Answer: A - Anorexia nervosa, binge-eating/purging type. The patient meets anorexia criteria including low weight, but also has recurrent binge/purge behavior.

Memory aid: *Low weight first: if anorexia criteria are met, binge/purge does not automatically mean bulimia.*

5. A therapist notes that Kemi has recurrent binge eating followed by compensatory behaviors such as vomiting, laxatives or excessive exercise, without the persistently low weight required for anorexia. Which condition should be considered first from the options below?

- A. Schizoid personality disorder
- B. Specific phobia
- C. Bulimia nervosa**
- D. Bipolar disorder

Answer: C - Bulimia nervosa. Bulimia is defined by binge episodes and inappropriate compensation; weight may be normal or above/below normal but not at the markedly low anorexia threshold.

Memory aid: *BULIMIA = Binge + Undo.*

6. Tina, a 30-year-old accountant, presents with recurrent binge eating with loss of control and distress but without regular compensatory purging or fasting. Which diagnosis or clinical concept best fits this presentation?

- A. Binge-eating disorder**
- B. Anorexia nervosa, binge-eating/purging type
- C. Substance use disorder
- D. Bulimia nervosa

Answer: A - Binge-eating disorder. Binge-eating disorder differs from bulimia because recurrent compensatory behaviors are absent.

Memory aid: *BED = Binge, no regular Erasing behavior.*

7. Samuel, a 30-year-old sales representative, presents with a problematic pattern of substance use causing clinically significant impairment or distress, including loss of control, risky use, social impairment or tolerance/withdrawal. Which diagnosis or clinical concept best fits this presentation?

- A. Borderline personality disorder
- B. Substance use disorder**
- C. Panic disorder
- D. Anorexia nervosa, restricting type

Answer: B - Substance use disorder. Substance use disorder is diagnosed from a maladaptive pattern and consequences, not simply from having used a substance.

Memory aid: *4C idea: Control loss, Consequences, Compulsion/craving, Continued use.*

8. A therapist notes that Daniel has initial euphoria followed by drowsiness or apathy, with slowed functioning and often constricted pupils. Which condition should be considered first from the options below?

- A. Opioid intoxication**
- B. Anorexia nervosa, restricting type
- C. Social anxiety disorder
- D. Specific phobia

Answer: A - Opioid intoxication. Opioids typically produce analgesia, euphoria followed by sedation and physiologic slowing.

Memory aid: *OPIOID = Often Pinpoint pupils, Inward/sedated, Opioid drowsiness.*

9. A therapist notes that Owen has euphoria, increased energy and confidence, decreased need for sleep, agitation and autonomic activation. Which condition should be considered first from the options below?

- A. Obsessive-compulsive disorder (OCD)
- B. Manic episode
- C. Stimulant intoxication**

D. Major depressive disorder

Answer: C - Stimulant intoxication. Stimulants activate rather than sedate; cocaine and amphetamines commonly produce energy, alertness and agitation.

Memory aid: *STIMULANT = STIMulates.*

10. During assessment, Wendy reports pervasive instability in relationships, self-image and affect, impulsivity, abandonment fears and recurrent self-harm or suicidal behavior. Which of the following is the BEST match?

A. Obsessive-compulsive personality disorder (OCPD)

B. Anorexia nervosa, restricting type

C. Borderline personality disorder

D. Obsessive-compulsive disorder (OCD)

Answer: C - Borderline personality disorder. Borderline personality disorder is marked by instability and intense reactivity across relationships, identity, mood and behavior.

Memory aid: *BORDER: Bonds unstable, Overreactive affect, Risky acts, Desertion fear, Empty/identity shifts, Recurrent self-harm.*

SECTION B: TRUE/FALSE - EXPLANATIONS

1. TRUE. Reduced need for sleep, grandiosity and pressured speech are compatible with mania.

Those are classic manic activation features.

2. FALSE. Bulimia nervosa necessarily requires markedly low body weight.

Markedly low weight is central to anorexia; bulimia can occur at many body weights.

3. TRUE. Binge-eating disorder is defined by recurrent binge eating without regular compensatory behaviors.

Regular purging or other compensation would point more toward bulimia if other criteria are met.

4. FALSE. OCPD and OCD are identical diagnoses.

OCPD is a personality pattern of perfectionism/control; OCD centers on obsessions and compulsions.

5. TRUE. Borderline personality disorder may include abandonment fears and recurrent self-injury.

Both are characteristic features of borderline personality disorder.

SECTION C: MODEL SHORT ANSWERS

1. Differentiate anorexia nervosa from bulimia nervosa. (2mks)

- Anorexia requires significantly low body weight with restriction/fear of weight gain/body-image disturbance.

- Bulimia centers on recurrent binge eating plus compensatory behavior; markedly low weight is not required.

Memory aid: *Low weight points strongly toward anorexia.*

2. State two characteristic features of a manic episode. (2mks)

- Reduced need for sleep, grandiosity, pressured speech, flight of ideas, increased goal-directed activity, risky behavior - any two.

Memory aid: *DIG FAST.*

3. What is comorbidity? (2mks)

- The presence of two or more disorders in the same person at the same time or over an overlapping period.

Memory aid: *CO-morbidity = conditions CO-occur.*

4. Name the four broad PTSD symptom clusters taught in modern formulations. (2mks)

- Intrusion

- Avoidance

- Negative changes in cognition/mood

- Arousal/reactivity

Memory aid: *IANA.*

5. What is meant by anhedonia? (2mks)

- Marked loss of interest or pleasure in activities previously enjoyed; a core depressive symptom.

Memory aid: *AN-hedonia = absence of pleasure.*

PART TWO: THEORIES/TECHNIQUES - ANSWERS

1. A therapist working with Brenda uses an intervention characterized by turning back onto oneself what one wants to do to another, or doing to oneself what one wishes another would do. Which approach or technique is being used?

A. Retroflexion

- B. Adlerian therapy
- C. In vivo exposure
- D. Dialectical Behavior Therapy (DBT)

Answer: A - Retroflexion. Retroflexion is a Gestalt contact-boundary disturbance directed back toward the self.

Memory aid: *RETRO = return action back to self.*

2. A therapist working with Wendy uses an intervention characterized by an approach emphasizing freedom, responsibility, choice, meaning, authenticity, isolation and awareness of mortality. Which approach or technique is being used?

- A. Person-centered therapy
- B. Interpretation
- C. Existential therapy**
- D. Psychoanalytic therapy

Answer: C - Existential therapy. Existential therapy helps clients face givens of existence and make authentic choices rather than simply trying to eliminate all anxiety.

Memory aid: *FMRC: Freedom, Meaning, Responsibility, Choice.*

3. In a session with Martha, the counselor emphasizes learning by association between stimuli so that a previously neutral cue comes to elicit a response. This BEST illustrates:

- A. Free association
- B. Classical conditioning**
- C. Existential therapy
- D. Extinction

Answer: B - Classical conditioning. Classical conditioning explains learned associations such as fear conditioning and underlies some exposure-based principles.

Memory aid: *Classical = cue-to-cue association.*

4. In a session with Uma, the counselor emphasizes behavior is shaped by its consequences, including reinforcement and punishment. This BEST illustrates:

- A. Person-centered therapy
- B. Operant conditioning**
- C. Psychoanalytic therapy
- D. Negative reinforcement

Answer: B - Operant conditioning. Operant conditioning focuses on consequences that increase or decrease behavior.

Memory aid: *Operant = behavior OPERATES, consequence follows.*

5. In a session with Amina, the counselor emphasizes a behavior increases because it removes or prevents an aversive stimulus. This BEST illustrates:

- A. Negative reinforcement**
- B. Person-centered therapy
- C. Retroflexion
- D. Interpretation

Answer: A - Negative reinforcement. Negative reinforcement strengthens behavior; negative means something unpleasant is removed, not that behavior is punished.

Memory aid: *Reinforcement always raises behavior; negative removes something.*

6. A therapist working with Irene uses an intervention characterized by a previously reinforced behavior decreases when the reinforcement maintaining it is consistently withheld. Which approach or technique is being used?

- A. Psychoanalytic therapy
- B. In vivo exposure
- C. Extinction**
- D. Social skills training

Answer: C - Extinction. Extinction occurs when reinforcement is no longer provided for a behavior.

Memory aid: *Extinction = extinguish the payoff.*

7. Which psychotherapy concept is most accurately described by the following: learning behavior by observing what happens to another person or by imitating a model?

- A. Empty-chair technique
- B. Social skills training
- C. Systematic desensitization
- D. Social modeling**

Answer: D - Social modeling. Modeling is observational learning: the client learns from another person's behavior and its consequences.

Memory aid: *MODEL = learn by watching a model.*

8. A therapist working with Fidel uses an intervention characterized by planned contact with feared stimuli, memories or situations while reducing avoidance so fear can be relearned. Which approach or technique is being used?

- A. Flooding
- B. Exposure therapy**
- C. In vivo exposure
- D. Existential therapy

Answer: B - Exposure therapy. Exposure reduces avoidance and allows corrective learning; it may be in vivo, imaginal or other formats.

Memory aid: *Approach, do not avoid.*

9. In a session with Uma, the counselor emphasizes direct, real-life exposure to a feared object or situation. This BEST illustrates:

- A. Adlerian therapy
- B. Gestalt dream work
- C. Rational emotive imagery
- D. In vivo exposure**

Answer: D - In vivo exposure. In vivo means exposure in the actual situation rather than only in imagination.

Memory aid: *VIVO = in real life.*

10. In a session with Grace, the counselor emphasizes gradual exposure to a fear hierarchy combined with relaxation or another incompatible response. This BEST illustrates:

- A. Resistance
- B. Cognitive behavioral therapy (CBT)
- C. Cognitive restructuring
- D. Systematic desensitization**

Answer: D - Systematic desensitization. Systematic desensitization classically combines relaxation training with stepwise movement through a fear hierarchy.

Memory aid: *RHE: Relax, Hierarchy, Expose gradually.*

11. In a session with Esther, the counselor emphasizes rapid and prolonged exposure to a high-intensity feared stimulus or situation without gradual hierarchy progression. This BEST illustrates:

- A. Flooding**

- B. Transference
- C. Extinction
- D. Classical conditioning

Answer: A - Flooding. Flooding differs from graded exposure because the client confronts a high level of fear more rapidly.

Memory aid: *Flooding = jump into the deep end.*

12. A therapist working with Martha uses an intervention characterized by systematically tensing and releasing muscle groups to learn physical relaxation and reduce arousal. Which approach or technique is being used?

- A. Gestalt therapy
- B. Progressive muscle relaxation**
- C. Extinction
- D. Psychoanalytic therapy

Answer: B - Progressive muscle relaxation. PMR builds awareness of tension and relaxation by working through muscle groups.

Memory aid: *Tense - notice - release - breathe.*

13. In a session with Tina, the counselor emphasizes a skills-based therapy especially associated with emotion dysregulation and borderline personality disorder, using mindfulness, distress tolerance, emotion regulation and interpersonal effectiveness. This BEST illustrates:

- A. Empty-chair technique
- B. Transference
- C. Retroflexion
- D. Dialectical Behavior Therapy (DBT)**

Answer: D - Dialectical Behavior Therapy (DBT). DBT combines acceptance and change strategies and teaches concrete skills for severe emotion dysregulation.

Memory aid: *M-D-E-I: Mindfulness, Distress tolerance, Emotion regulation, Interpersonal effectiveness.*

14. In a session with Irene, the counselor emphasizes behavioral teaching and rehearsal of interpersonal skills such as greetings, eye contact, listening and conversation. This BEST illustrates:

- A. Social skills training**
- B. Cognitive behavioral therapy (CBT)
- C. Transference
- D. Flooding

Answer: A - Social skills training. Social skills training uses instruction, modeling, rehearsal and feedback to build interpersonal behavior.

Memory aid: *IMRF: Instruction, Modeling, Rehearsal, Feedback.*

15. Which psychotherapy concept is most accurately described by the following: the client says whatever thoughts, feelings, images or memories come to mind without censoring them?

- A. Free association**
- B. Psychoanalytic therapy
- C. Operant conditioning
- D. Countertransference

Answer: A - Free association. Free association is a classic psychoanalytic technique intended to reduce censorship and reveal unconscious material.

Memory aid: *Psychoanalytic FIRT: Free association, Interpretation, Resistance, Transference.*

SECTION B: TRUE/FALSE - EXPLANATIONS

1. FALSE. Negative reinforcement weakens behavior.

All reinforcement strengthens behavior; negative reinforcement does so by removing an aversive condition.

2. TRUE. Extinction involves withholding the reinforcement that has maintained a behavior.

When reinforcement stops, the behavior may gradually decrease.

3. TRUE. Systematic desensitization classically combines relaxation with graded exposure.

The client learns relaxation and moves through a fear hierarchy.

4. FALSE. Flooding is always more gradual than systematic desensitization.

Flooding is deliberately high-intensity and comparatively rapid.

5. TRUE. Gestalt therapy emphasizes present-moment awareness.

Here-and-now awareness is central to Gestalt work.

SECTION C: MODEL SHORT ANSWERS

1. State two core conditions of person-centered therapy. (2mks)

- Any two: accurate empathy, unconditional positive regard, congruence/genuineness.

Memory aid: EUC.

2. Give the ABC elements of REBT. (2mks)

- A = Activating event

- B = Belief about the event

- C = emotional/behavioral Consequence

Memory aid: ABC: event does not mechanically create emotion; beliefs mediate.

3. Explain negative reinforcement with an example. (1mks)

- Negative reinforcement increases a behavior by removing an aversive stimulus.

- Example: fastening a seat belt to stop the warning sound makes future seat-belt fastening more likely.

Memory aid: Reinforcement increases; negative removes.

PART THREE: ETHICAL/LEGAL - ANSWERS

1. A counselor asks which principle or professional concept is defined by a departure from customary therapeutic boundaries that may be clinically appropriate and non-exploitative in context. The BEST answer is:

- A. Ethical response to colleague misconduct
- B. Record keeping
- C. Boundary crossing**
- D. Dual or multiple relationship

Answer: C - Boundary crossing. A boundary crossing is not automatically unethical; context, culture, intent and effect matter.

Memory aid: *Crossing may be thoughtful; violation causes or risks serious harm/exploitation.*

2. In professional practice, a situation calls for a serious harmful or exploitative breach of therapeutic boundaries. Which ethical/legal concept is MOST directly involved?

- A. Boundary crossing
- B. Gift decision-making
- C. Technology ethics
- D. Boundary violation**

Answer: D - Boundary violation. Boundary violations exploit the therapeutic relationship or cause significant harm and are unethical.

Memory aid: *Violation = harm/exploitation.*

3. In professional practice, a situation calls for sexual or romantic involvement with a current therapy client. Which ethical/legal concept is MOST directly involved?

- A. Beneficence
- B. Gift decision-making
- C. Mandated reporting
- D. Sexual relationship with a current client**

Answer: D - Sexual relationship with a current client. Sexual relationships with current clients are prohibited because of power imbalance and exploitation risk. Terminating therapy to start a relationship does not erase those concerns.

Memory aid: *Current client + sex = no.*

4. A counselor asks which principle or professional concept is defined by considering clinical meaning, value, timing, culture, power and exploitation risk before accepting or declining a client gift. The BEST answer is:

- A. Nonmaleficence
- B. Gift decision-making**
- C. Duty to warn/protect
- D. Sexual relationship with a current client

Answer: B - Gift decision-making. Gifts require contextual judgment. A culturally meaningful modest gift may be handled differently from an expensive or coercive gift.

Memory aid: *VALUE: Value, Understanding/meaning, Local culture, Unequal power, Effect on therapy.*

5. In professional practice, a situation calls for accepting goods or services instead of money when ethically permissible, not exploitative and not clinically contraindicated. Which ethical/legal concept is MOST directly involved?

- A. Duty to warn/protect
- B. Privacy
- C. Fidelity
- D. Bartering**

Answer: D - Bartering. Bartering may be acceptable under some codes when not exploitative and when arrangements are clear and clinically appropriate.

Memory aid: *Barter only if Boundaries and Benefits are safe.*

6. During supervision, Hassan describes maintaining accurate, timely, secure and clinically appropriate client records. Which concept should guide the discussion?

- A. Confidentiality
- B. Record keeping**
- C. Boundary violation
- D. Privacy

Answer: B - Record keeping. Records support continuity of care, accountability and legal/ethical duties; access must be controlled and retention rules followed.

Memory aid: ACTS: Accurate, Current, Timely, Secure.

7. During supervision, Kemi describes ending therapy responsibly with preparation, continuity planning and referral when needed rather than abandoning the client. Which concept should guide the discussion?

- A. Record keeping
- B. Bartering
- C. Termination and referral**
- D. Fidelity

Answer: C - Termination and referral. Ethical termination includes adequate notice, discussion, referrals or continuity plans when appropriate.

Memory aid: PLAN: Prepare, Link/referral, Address feelings, Next steps.

8. In professional practice, a situation calls for using ethical codes, consultation/supervision and appropriate reporting procedures when credible serious misconduct is observed. Which ethical/legal concept is MOST directly involved?

- A. Informed consent
- B. Ethical response to colleague misconduct**
- C. Sexual relationship with a current client
- D. Beneficence

Answer: B - Ethical response to colleague misconduct. Serious ethical violations should not simply be ignored; follow code, organizational and legal procedures while protecting clients and due process.

Memory aid: C-A-R-E: Consult code, Assess facts, Report appropriately, Ensure client safety.

9. A counselor asks which principle or professional concept is defined by protecting privacy, competence, informed consent, identity, emergency planning and data security when using digital or remote services. The BEST answer is:

- A. Informed consent
- B. Privileged communication
- C. Autonomy
- D. Technology ethics**

Answer: D - Technology ethics. Technology changes the medium, not the professional duty. Security, consent and contingency planning remain essential.

Memory aid: SPACE: Security, Privacy, Access/identity, Consent, Emergency plan.

10. In professional practice, a situation calls for not automatically disclosing the entire file on receipt of a subpoena; consult applicable law, the client/attorney and professional guidance, and disclose only as authorized or legally required. Which ethical/legal concept is MOST directly involved?

- A. Subpoena response**
- B. Ethical response to colleague misconduct
- C. Fidelity
- D. Bartering

Answer: A - Subpoena response. A subpoena and a court order are not always identical. The safest course-based principle is not to reflexively release all records; seek proper legal/ethical guidance and protect confidentiality to the extent permitted.

Memory aid: STOP: See legal guidance, Tell/consult client, Only required information, Protect confidentiality.

11. During supervision, Brenda describes respecting the client's right to make informed choices about treatment. Which concept should guide the discussion?

- A. Subpoena response
- B. Veracity
- C. Confidentiality

D. Autonomy

Answer: D - Autonomy. Autonomy supports informed choice, consent and respect for the client's self-determination.

Memory aid: A = Allow informed choice.

12. A counselor asks which principle or professional concept is defined by acting to promote the client's welfare and benefit. The BEST answer is:

- A. Duty to warn/protect
- B. Bartering
- C. Privileged communication

D. Beneficence

Answer: D - Beneficence. Beneficence asks the professional to actively contribute to the client's well-being.

Memory aid: BENEFIT = beneficence.

13. In professional practice, a situation calls for avoiding or minimizing harm. Which ethical/legal concept is MOST directly involved?

- A. Bartering
- B. Technology ethics

C. Nonmaleficence

D. Confidentiality

Answer: C - Nonmaleficence. Nonmaleficence is the duty to do no harm and to avoid exposing clients to unnecessary risk.

Memory aid: NON-maleficence = NO harm.

14. A counselor asks which principle or professional concept is defined by fair and equitable distribution of services, benefits, burdens and opportunities. The BEST answer is:

- A. Professional competence
- B. Record keeping
- C. Subpoena response

D. Justice

Answer: D - Justice. Justice concerns fairness and equitable treatment, including access and resource allocation.

Memory aid: Justice = fairness.

15. A counselor asks which principle or professional concept is defined by truthfulness and honest communication with clients. The BEST answer is:

- A. Veracity**
- B. Gift decision-making
- C. Record keeping
- D. Dual or multiple relationship

Answer: A - Veracity. Veracity requires accurate, truthful information rather than deception.

Memory aid: VERACITY = verifiable truth.

SECTION B: MODEL SHORT ANSWERS

1. Give two elements of informed consent. (2mks)

- Examples: nature/purpose of services, risks, benefits, alternatives, fees, therapist qualifications, limits of confidentiality, voluntary choice, right to ask questions.

Memory aid: BRALC.

2. When is termination ethically appropriate? (1mks)

- When goals are met, treatment is no longer beneficial/needed, the therapist cannot provide competent service, or other legitimate reasons exist - with adequate planning to avoid abandonment.

Memory aid: PLAN.

3. State one key ethical requirement for clinical records. (1mks)

- Records should be accurate, timely, relevant and securely stored with access protected.

Memory aid: ACTS.

4. Why are dual relationships ethically risky? (1mks)

- They can impair objectivity or professional judgment.
- They can increase risk of exploitation, coercion, conflicts of interest or harm.

Memory aid: Power + objectivity + exploitation.

SECTION C: MODEL SHORT ESSAYS

1. What is a boundary violation? (2mks)

- A serious harmful or exploitative breach of therapeutic boundaries.

Memory aid: Violation = harm/exploitation.

2. State two technology-related ethical concerns in teletherapy. (2mks)

- Examples: data security/privacy, informed consent, identity verification, competence with technology, emergency location/plan, jurisdiction.

Memory aid: SPACE.

3. How should a therapist respond to a subpoena for records? (1mks)

- Do not reflexively release the entire file.
- Consult applicable law/professional guidance and the client/attorney; protect confidentiality and disclose only what is authorized or legally required.

Memory aid: STOP.

PART FOUR: CULTURE / GROUP / COUNSELING PROCESS / TREATMENT PLAN - ANSWERS

1. A trainee observes creating safety, trust, empathy and collaborative engagement at the beginning of counseling. This is an example of:

- A. Rapport building**
- B. Cultural competence
- C. Cognitive-behavioral group
- D. Psychoeducational group

Answer: A - Rapport building. Rapport supports disclosure and collaboration; it begins early but continues throughout treatment.

Memory aid: *RAPPORT starts with Respect, Attention, Presence.*

2. A trainee observes systematically exploring the client's problems, symptoms, context, history, strengths, risk and functioning. This is an example of:

- A. Group role
- B. Assessment**
- C. Termination and follow-up
- D. Active listening

Answer: B - Assessment. Assessment precedes or accompanies formulation and goal setting; it includes both problems and resources.

Memory aid: *HRSF: History, Risk, Symptoms, Functioning.*

3. Which counseling/group/cultural concept is BEST described by: collaboratively defining client-centered outcomes that are realistic and observable?

- A. Norming stage
- B. Storming stage
- C. Open-ended questioning
- D. Goal setting**

Answer: D - Goal setting. Goals are stronger when the client participates and the goals are specific enough to evaluate.

Memory aid: *SMART: Specific, Measurable, Achievable, Realistic/Relevant, Time-bound.*

4. A trainee observes implementing agreed therapeutic strategies and monitoring the client's response. This is an example of:

- A. Intervention stage**
- B. Professional interpreter use
- C. Self-disclosure
- D. Norming stage

Answer: A - Intervention stage. Intervention follows assessment and goal definition and should be linked to the case formulation and treatment plan.

Memory aid: *Assess -> Goal -> Intervene -> Review.*

5. A trainee observes reviewing progress, consolidating coping skills, planning relapse prevention or referral and ending the professional relationship responsibly. This is an example of:

- A. Progress review
- B. Rapport building
- C. Termination and follow-up**
- D. Group confidentiality

Answer: C - Termination and follow-up. Termination is a therapeutic phase, not simply stopping appointments.

Memory aid: *RECAP: Review, Evaluate, Consolidate, Aftercare, Part.*

6. In counseling practice, Xavier encounters a situation involving attending closely to verbal and nonverbal communication and showing the client that the message has been heard and understood. Which concept BEST applies?

- A. Performing stage
- B. Active listening**

- C. Rapport building
- D. Cultural imposition

Answer: B - Active listening. Active listening includes presence, minimal encouragers, reflection, clarification and appropriate nonverbal attending.

Memory aid: SOLER: *Squarely face, Open posture, Lean, Eye contact, Relaxed.*

7. A trainee observes identifying and feeding back the emotional meaning expressed by the client. This is an example of:

- A. Reflection of feeling**
- B. Self-disclosure
- C. Adjourning stage
- D. Skills-development group

Answer: A - Reflection of feeling. Reflection of feeling helps clients recognize and explore emotions rather than only facts.

Memory aid: *Name the feeling, not just the facts.*

8. A trainee observes restating the essence of the client's content in fresh words without changing its meaning. This is an example of:

- A. Forming stage
- B. Paraphrasing**
- C. Cognitive-behavioral group
- D. Self-disclosure

Answer: B - Paraphrasing. Paraphrasing checks understanding and encourages elaboration.

Memory aid: *Paraphrase = same meaning, new words.*

9. A trainee observes questions that invite elaboration rather than a yes/no answer. This is an example of:

- A. Reflection of feeling
- B. Support group
- C. Group confidentiality
- D. Open-ended questioning**

Answer: D - Open-ended questioning. Open questions support exploration, while excessive questioning can turn therapy into an interrogation.

Memory aid: *WHAT/HOW usually open more than DID/IS.*

10. Which counseling/group/cultural concept is BEST described by: respectfully pointing out discrepancies between a client's words, behaviors, values or nonverbal cues?

- A. Support group
- B. Performing stage
- C. Cognitive-behavioral group
- D. Therapeutic confrontation**

Answer: D - Therapeutic confrontation. Confrontation is not aggression; it is a careful invitation to examine inconsistency.

Memory aid: *Confront discrepancy, not the person.*

11. In counseling practice, Amina encounters a situation involving the therapist deliberately shares limited personal information when it is likely to benefit the client rather than meet the therapist's own needs. Which concept BEST applies?

- A. Self-disclosure**
- B. Cognitive-behavioral group
- C. Paraphrasing
- D. Active listening

Answer: A - Self-disclosure. Self-disclosure should be purposeful, brief and client-centered.

Memory aid: *WAIT: Why am I telling this?*

12. Which counseling/group/cultural concept is BEST described by: a collaborative written or explicit plan linking identified problems to goals, measurable objectives, interventions and review?

- A. Intervention stage
- B. Norming stage
- C. Assessment

D. Treatment plan

Answer: D - Treatment plan. Treatment plans organize care around the client's specific needs and should be revised according to progress.

Memory aid: PGOIR: Problem, Goal, Objectives, Interventions, Review.

13. Which counseling/group/cultural concept is BEST described by: an organized explanation linking the client's presenting problems with relevant history, triggers, maintaining factors, strengths and a rationale for intervention?

- A. Case formulation**
- B. Goal setting
- C. Forming stage
- D. Ethnocentrism

Answer: A - Case formulation. Formulation goes beyond a diagnosis by explaining why this person is struggling in this way at this time and what may help.

Memory aid: 5P option: Presenting, Predisposing, Precipitating, Perpetuating, Protective.

14. Which counseling/group/cultural concept is BEST described by: evaluating whether objectives are being achieved and modifying the treatment plan when progress is absent, circumstances change or goals are met?

- A. Professional interpreter use
- B. Skills-development group
- C. Paraphrasing

D. Progress review

Answer: D - Progress review. Treatment planning is dynamic; review links evidence of progress to continuation, modification or termination.

Memory aid: Review = measure, discuss, revise.

15. Which counseling/group/cultural concept is BEST described by: ongoing development of awareness, cultural knowledge, clinical skills and meaningful encounters with diverse clients?

- A. Norming stage
- B. Cultural competence**
- C. Forming stage
- D. Professional interpreter use

Answer: B - Cultural competence. Cultural competence requires reflective awareness plus knowledge, skills and real encounters; it is not achieved by assuming everyone should conform to the therapist's culture.

Memory aid: AKSE: Awareness, Knowledge, Skills, Encounters.

SECTION B: MODEL SHORT ESSAYS

1. Differentiate psychoeducational groups from skills-development groups. (4mks)

- Psychoeducational groups emphasize teaching information/understanding about a problem or condition.
- Skills groups emphasize practicing specific behaviors or coping/interpersonal skills with rehearsal and feedback.

Memory aid: Education = know; skills = do.

2. Define a treatment plan and list four common elements. (4mks)

- A collaborative roadmap tailored to the client linking assessment/formulation to care.
- Common elements: identified problem(s), goals, measurable objectives, interventions/strategies, responsible persons/frequency as relevant, review/progress criteria.

Memory aid: PGOIR.

3. Explain SMART goals. (2mks)

- Specific
- Measurable
- Achievable
- Relevant/Realistic
- Time-bound

Memory aid: SMART.

SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

Session: Revision Mock Set 02	Duration: 1hr 30mins
Specialty: Psychotherapy	Coefficient: 04
Eliminatory mark: < 14/20	Training reference: VTI-AEDA examination style

SIMULATION OF REAL WORKING SITUATION PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 02

Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.

DETAILED MODEL ANSWERS - SIMULATION OF REAL WORKING SITUATIONS

SECTION I

1. Generalized anxiety and worry - anchor: GAD - CBT with cognitive restructuring, worry management and relaxation

Sasha, 59, worries most days about finances, health, family and work. The worry has persisted for many months, is difficult to stop, and is accompanied by muscle tension, restlessness and poor sleep. she repeatedly seeks reassurance but remains anxious.

Model response / marking points (25 marks):

1. Assess breadth, duration and controllability of worry and rule out substance/medical contributors.
2. Formulate the cycle of threat prediction, reassurance/avoidance and short-lived relief.
3. Use psychoeducation about anxiety and uncertainty.
4. Identify probability overestimation and catastrophic beliefs.
5. Use cognitive restructuring and behavioral experiments.
6. Teach breathing or progressive muscle relaxation for arousal.
7. Consider scheduled worry/problem-solving time and reduction of reassurance seeking.
8. Set graded behavioral goals and homework.
9. Review sleep, caffeine/substance use and supports.
10. Monitor outcome and adjust plan.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: GAD = General Anxiety across Domains; CBT = Catch, Check, Change.

2. Panic with avoidance - anchor: Panic disorder with agoraphobic avoidance - CBT and exposure

Komiko, 18, has recurrent sudden episodes of racing heart, sweating, breathlessness and fear of dying. After several attacks in public places, he now avoids buses, markets and long queues and only goes out with a companion.

Model response / marking points (25 marks):

1. Assess attack features, medical/substance causes, frequency, anticipatory anxiety and avoidance.
2. Explain the panic cycle and misinterpretation of bodily sensations.
3. Teach realistic reappraisal of sensations.
4. Use interoceptive exposure where appropriate to feared bodily sensations.
5. Build graded in vivo exposure to avoided settings.
6. Reduce safety behaviors gradually.
7. Use breathing skills carefully as coping, not as proof that anxiety is dangerous.

8. Track predictions and actual outcomes.
9. Assess risk and comorbidity.
10. Plan relapse prevention.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: *Panic: BODY sensation -> catastrophic THOUGHT -> FEAR -> avoidance; exposure breaks the loop.*

SECTION II

1. OCD contamination and washing - anchor: CBT with Exposure and Response Prevention (ERP)

Wendy, 25, spends several hours each day washing hands and cleaning after intrusive fears of contamination. she knows the rituals are excessive but believes that not washing could cause serious harm to family members.

Model response / marking points (25 marks):

1. Clarify obsessions, compulsions, triggers, time burden and insight.
2. Explain how rituals reduce anxiety briefly and thereby maintain OCD.
3. Develop a graded exposure hierarchy collaboratively.
4. Expose to feared contamination cues in planned steps.
5. Prevent or delay the washing ritual during/after exposure.
6. Remain long enough for new learning rather than immediate escape.
7. Address inflated responsibility/catastrophic beliefs with cognitive methods as needed.
8. Do not provide repeated reassurance that becomes part of the compulsion.
9. Use homework and repeated practice.
10. Monitor distress, safety and progress.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: *ERP = Expose + Resist ritual + Practice.*

2. Social anxiety and skills practice - anchor: Social anxiety - CBT, graded exposure and social skills training if deficits are present

Mark, 32, avoids speaking in meetings and meeting new people because of fear of blushing, stumbling over words and being judged as incompetent. This is beginning to limit career and friendships.

Model response / marking points (25 marks):

1. Assess feared situations, predictions, avoidance and actual skill deficits.
2. Explain self-focused attention and avoidance/safety behaviors.
3. Identify negative predictions about judgment.
4. Build graded social/performance exposures.
5. Use behavioral experiments and reduce safety behaviors.
6. If skill deficits exist, use instruction, modeling, role play, rehearsal and feedback.
7. Practice attention shifting toward the interaction.
8. Set homework in real social situations.
9. Review outcomes against predictions.
10. Plan maintenance and relapse prevention.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound

alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: *IMRF for skills: Instruction, Modeling, Rehearsal, Feedback.*